Master Prompt: Complete Hospital Architectural Design Proposal

Community Hospital for Pasquilla–Mochuelo, Ciudad Bolívar, Bogotá, Colombia

Architectural design, healthcare planning, site feasibility and technical proposal — Healthcare architecture · Colombia · Evidence-based design

How to use this prompt

- Copy the complete prompt below into Claude.
- Replace the project-input fields with your available information.
- Upload a site map, satellite image, cadastral plan or survey if available.
- Ask Claude to complete the proposal in phases, validating each phase before proceeding to detailed design.

Project input checklist

1. **Site information** — Google Maps pin, cadastral plan, plot boundaries, survey or satellite image.
2. **Project purpose** — Academic design project, government proposal, NGO project or actual investment.
3. **Hospital capacity** — Expected consultations, emergency services, observation or inpatient beds, if known.
4. **Site area** — Verified land area, or allow the feasibility stage to establish the requirement.
5. **Budget** — Budget in COP, funding source or a requirement for preliminary cost scenarios.
6. **Local planning data** — Land-use classification, terrain information, existing healthcare facilities and population data.
7. **Drawing requirements** — Required plans, scales, presentation sheets and software.

Recommended workflow

1. Establish the need and select the site: verify healthcare access, compare candidate parcels and check land-use and environmental restrictions. This is the most important step because the exact plot is not yet established.
2. Establish the clinical brief and area programme: determine services, room quantities, room areas, patient flows and operational requirements before fixing the building size.
3. Develop and compare architectural concepts: alternative site layouts, massing studies, circulation diagrams and functional concepts before selecting a design.
4. Develop coordinated drawings and technical systems: plans, sections, elevations, structure, services, accessibility, infection control, sustainability and preliminary costs.

> **Recommendation:** Start with the feasibility stage, not the floor plans. For Pasquilla–Mochuelo, the architectural concept should follow the evidence on healthcare access, suitable land and the appropriate service level. Otherwise, you risk producing an impressive hospital design on a site that may not be suitable or a facility that duplicates existing services.

MASTER PROMPT: COMPLETE ARCHITECTURAL DESIGN PROPOSAL FOR A COMMUNITY HOSPITAL IN BOGOTÁ, COLOMBIA

1. Your Role

Act as an integrated multidisciplinary architectural and healthcare consultancy comprising: Principal Architect; Healthcare Architect and Hospital Planner; Urban Designer and Regional Planner; Site Selection and Feasibility Consultant; Healthcare Operations Planner; Medical Planning Consultant; Structural Engineer; Geotechnical Engineer; Civil and Infrastructure Engineer; Mechanical, Electrical and Plumbing (MEP) Engineer; Medical Gas Systems Consultant; Fire and Life Safety Consultant; Infection Prevention and Control Consultant; Environmental and Sustainability Consultant; Landscape Architect; Accessibility Consultant; Quantity Surveyor and Cost Estimator; BIM Coordinator; CAD Documentation Specialist; Architectural Visualisation Artist; Colombian Building Regulations Researcher; Healthcare Project Manager.

Your task is to develop a comprehensive, technically reasoned, evidence-based architectural design proposal for a proposed community hospital in Bogotá, Colombia.

Do not produce merely a conceptual description, generic hospital layout, mood board or collection of architectural renders. Develop a coordinated proposal progressing from project justification and site feasibility to the architectural brief, area programming, conceptual design, functional planning, technical systems, cost assessment and final presentation.

Distinguish verified facts, preliminary assumptions, professional recommendations and items requiring confirmation by local authorities or consultants.

2. Project Information

Project title: Proposed Community Hospital for Improved Healthcare Access in Pasquilla–Mochuelo, Ciudad Bolívar, Bogotá, Colombia.

Project location

- Country: Colombia
- City: Bogotá, Capital District
- Locality: Ciudad Bolívar
- Preliminary study area: Pasquilla–Mochuelo rural–urban transition
- General site context: Southern Bogotá, including rural settlements and surrounding neighbourhoods
- Initial map references:
    - Pasquilla: https://www.google.com/maps/search/Pasquilla,+Ciudad+Bolivar,+Bogota,+Colombia
    - Mochuelo: https://www.google.com/maps/search/Mochuelo,+Ciudad+Bolivar,+Bogota,+Colombia

Critical site-selection qualification

No exact plot has been selected or verified. The project area is a candidate for investigation, not an officially confirmed hospital site.

Do not invent: a cadastral parcel or property boundary; exact site coordinates; land ownership; plot area or dimensions; existing utility capacity; officially established healthcare shortages; population figures without a source;

hospital travel times without a network analysis; zoning permissions or environmental approvals; confirmation that the proposed land is vacant or buildable.

First establish whether the area demonstrates a measurable healthcare accessibility gap and whether a suitable parcel can legally and physically accommodate the proposed facility.

If an exact plot cannot be established, develop a preliminary site-selection framework and clearly label any hypothetical site used for concept development.

Project purpose

Investigate and develop an appropriate healthcare facility that could improve access to primary healthcare, diagnostics, maternal and child healthcare, urgent assessment and emergency stabilisation for residents in underserved parts of the study area, subject to evidence of local demand.

Initial facility hypothesis

Evaluate a community-oriented, primary-care-led healthcare facility with urgent-care capability. Consider, subject to healthcare demand and referral arrangements: general outpatient consultation; nursing and preventive healthcare; maternal and child health; basic laboratory services; essential diagnostic imaging; pharmacy; minor procedures and treatment; emergency assessment and stabilisation; observation beds if justified; ambulance access and referral coordination; telemedicine and community outreach.

Do not automatically propose a large tertiary hospital, major surgical centre, intensive care unit, or inpatient ward. Establish the appropriate service level from documented demand, the existing hospital network, referral times, staffing, operational viability and regulatory requirements.

3. Project Inputs and Assumptions

Use the following fields as the initial project brief.

Field	Value
Project type	New healthcare facility
Project stage	Feasibility study and preliminary architectural design
Client	[INSERT CLIENT OR ACADEMIC PROJECT CLIENT]
Client’s objective	[INSERT OBJECTIVE OR DEVELOP A PROVISIONAL OBJECTIVE]
Intended users	Residents of the verified service catchment, patients, visitors, clinicians, support staff and emergency personnel
Target catchment	[INSERT VERIFIED POPULATION OR MARK AS TO BE ESTABLISHED]
Required clinical capacity	[INSERT OR DETERMINE THROUGH FEASIBILITY]
Bed capacity	[INSERT OR JUSTIFY WHETHER INPATIENT BEDS ARE REQUIRED]
Plot area	[INSERT VERIFIED AREA OR MARK AS UNKNOWN]
Plot coordinates	[INSERT VERIFIED COORDINATES OR MARK AS UNKNOWN]

Field	Value
Land ownership	[INSERT OR VERIFY]
Construction budget	[INSERT CURRENCY AND BUDGET OR MARK AS UNKNOWN]
Delivery timeline	[INSERT OR PROPOSE A PRELIMINARY PROGRAMME]
Architectural approach	Climate-responsive, context-sensitive, accessible, human-centred and operationally efficient
Preferred deliverables	Written report, analytical diagrams, area schedule, bubble diagrams, site plan, floor plans, sections, elevations, design-development diagrams, technical strategy, visualisations and preliminary cost plan
Design software	[INSERT AVAILABLE SOFTWARE, SUCH AS REVIT, ARCHICAD, AUTOCAD, QGIS, SKETCHUP OR RHINO]

When information is missing, do not stop unnecessarily. Create a clearly labelled assumption register, explain how each assumption affects the proposal and identify the investigation required to resolve it.

4. Research and Evidence Requirements

Conduct source-based research before making design decisions. Prioritise official Colombian and Bogotá sources:

1. Bogotá District Health Secretariat: https://www.saludcapital.gov.co/
2. Bogotá Open Data Portal: https://datosabiertos.bogota.gov.co/
3. Bogotá District Planning Secretariat: https://www.sdp.gov.co/
4. Colombia's National Health Superintendence: https://www.supersalud.gov.co/
5. Bogotá's official geographical and territorial information resources, including relevant IDECA datasets.
6. Official Bogotá land-use, territorial planning and risk-management information.
7. Colombian Ministry of Health and Social Protection: https://www.minsalud.gov.co/
8. Colombian National Disaster Risk Management Unit: https://portal.gestiondelriesgo.gov.co/
9. Colombian geological and geotechnical information from the relevant competent authorities.
10. Applicable Colombian building, accessibility, structural, seismic, fire safety, environmental, sanitation and healthcare-facility requirements.

Use official datasets, regulations, technical publications and reliable healthcare planning literature.

For every important factual claim, provide: claim or finding; source title; publishing authority; publication or update date, if available; direct URL; date accessed; how the finding affects the design.

Do not fabricate references, statistics, legal requirements or URLs. If a source cannot be accessed, identify it as requiring verification. Clearly distinguish current legal requirements from international best practices and voluntary design recommendations.

5. Phase 1 — Project Justification and Healthcare Need

Establish why the proposed project may be necessary.

5.1 Regional healthcare context. Investigate: Ciudad Bolívar's urban and rural settlement patterns; distribution of residents and population density; existing healthcare facilities and their actual service levels; public and private providers; existing primary care centres, clinics and hospitals; available outpatient, emergency, diagnostic, maternity and inpatient services; public transport connectivity; road network and ambulance access; geographic barriers and difficult terrain; socioeconomic and demographic characteristics relevant to healthcare access; existing referral pathways.

5.2 Healthcare accessibility analysis. Create an inventory of existing healthcare facilities using official or otherwise reliable data. For each facility, record, where available: name; coordinates; facility type; service capability; operating hours; emergency capability; bed capacity; public or private status; source and verification date.

Analyse access using road-network travel times, not straight-line distances alone. If adequate data are available, compare walking, public transport, private vehicle and ambulance access; peak and off-peak travel; and access during adverse weather or road disruption, where evidence permits. Map population clusters and identify areas with poor access to the relevant level of healthcare. Do not assume that the nearest building provides the required service. Distinguish proximity from actual clinical capability.

5.3 Service gap assessment. Determine whether the principal need is for:

- A. A primary healthcare centre.
- B. A primary healthcare facility with urgent stabilisation.
- C. A community hospital with limited inpatient care.
- D. An alternative intervention, such as upgrading an existing facility, mobile healthcare or improving transport and referral systems.

Compare these options using transparent criteria: unmet demand; accessibility improvement; clinical scope; workforce availability; capital expenditure; operating expenditure; referral efficiency; environmental impact; implementation feasibility.

Required deliverable. A project justification report with an evidence table, a healthcare facility map, a preliminary accessibility analysis and a reasoned recommendation on the appropriate facility type. If available evidence does not demonstrate the need for a new hospital, explicitly recommend further study or an alternative intervention rather than manufacture a justification.

6. Phase 2 — Site Selection and Feasibility

Compare several candidate locations within the Pasquilla–Mochuelo study area. Do not select a precise parcel until reliable geographic, legal and physical information is available.

6.1 Site-selection criteria. Assess: healthcare accessibility gap; catchment population; road access; ambulance access; public transport; pedestrian accessibility; land-use compatibility; land ownership and acquisition feasibility; plot geometry; developable area; terrain and slope; geotechnical stability; flood and landslide susceptibility; drainage; water availability; wastewater infrastructure; electricity supply; telecommunications; environmental protection restrictions; nearby incompatible uses; air quality and noise; construction logistics; emergency access and evacuation; scope for future expansion.

6.2 GIS and spatial analysis. Where data are available, use QGIS or equivalent GIS tools to prepare: study-area location map; existing healthcare facility distribution map; population density map; healthcare catchment and accessibility map; road and public transport map; terrain and slope map; environmental constraint map; land-use and zoning map; flood and landslide hazard screening map; candidate parcel comparison map. Identify dataset

resolution, date, coordinate reference system and limitations. Do not infer parcel-level suitability from coarse regional data.

6.3 Weighted site-selection matrix. Develop a transparent multi-criteria decision matrix. For each criterion, provide: weight; scoring scale; evidence used; score for each candidate; weighted score; confidence level; reason for the score. Explain the weighting method and test whether reasonable changes in weights alter the preferred site. Do not use invented precision. If evidence is unavailable, mark the score as unknown and identify the information required.

Required deliverable. A comparative site-selection report identifying: preferred candidate, if evidence supports one; alternative candidate; reasons for selection; critical unresolved constraints; surveys, approvals and investigations needed before site confirmation.

7. Phase 3 — Site and Climate Analysis

After establishing a verified or explicitly hypothetical site, develop a detailed architectural site analysis. Investigate: geographic coordinates and elevation; site boundaries and adjoining properties; north direction; contours and slope gradients; existing vegetation and structures; neighbourhood character; road hierarchy; access points; pedestrian movement; public transport stops; existing views; noise sources; solar exposure; wind conditions; rainfall and drainage; temperature and humidity; local microclimate; water availability; environmental sensitivities; construction access; emergency vehicle movements.

For Bogotá's high-altitude Andean setting, investigate relevant temperature variation, rainfall patterns, solar radiation, orientation, wind exposure and thermal comfort. Do not apply a generic tropical or hot-climate strategy without checking local climate data.

Required analytical drawings: (1) regional context map; (2) local context map; (3) site location plan; (4) site photographs or documented visual references; (5) existing land-use map; (6) access and movement diagram; (7) contour and slope analysis; (8) sun and shadow analysis; (9) climate response diagram; (10) environmental constraints diagram; (11) opportunities and constraints diagram. If exact survey data are unavailable, clearly label diagrams as preliminary.

8. Phase 4 — Client Objectives and Design Brief

Develop a clear statement of the client's objectives. Separate: clinical, patient experience, staff operational, community, architectural, environmental, financial, regulatory and future expansion objectives. Convert each objective into a measurable or verifiable design response.

Create an **objective-to-design matrix** containing: (1) client objective; (2) identified problem; (3) design response; (4) performance indicator; (5) verification method.

Proposed design principles. Investigate the suitability of: patient-centred design; dignified and non-institutional environments; clear wayfinding; separation of public and clinical circulation where required; infection prevention; universal accessibility; privacy and confidentiality; staff wellbeing; low-maintenance materials; resilience and continuity of care; flexible clinical spaces; future expansion; appropriate integration with the surrounding landscape.

Do not assume all principles require separate departments or expensive architectural features. Prioritise operationally meaningful interventions.

9. Phase 5 — Healthcare Service Model and Capacity

Develop a clinically coherent service model before fixing the floor area.

9.1 Define the service level. Recommend a facility category based on: population demand; disease burden, where reliable data exist; existing healthcare provision; referral arrangements; clinical staffing; operating costs; regulatory classification; required diagnostic and treatment services.

9.2 Define clinical capacity. Estimate, with explicit assumptions: daily outpatient consultations; peak hourly arrivals; nursing and preventive care workload; maternal and child health demand; laboratory workload; imaging demand; emergency arrivals; observation requirements; staff numbers and shift patterns; waiting and support requirements. Where sufficient data are available, use transparent calculations (for example, consultation rooms from daily appointments, room utilisation, working hours and average consultation duration). For emergency and observation spaces, use appropriate healthcare planning methods rather than simply multiplying population by a generic bed ratio.

9.3 Healthcare departments. Evaluate the inclusion and scale of:

- **A. Public and administrative:** entrance lobby; reception and registration; waiting areas; patient information; administration; staff facilities; public toilets; security.
- **B. Outpatient care:** general consultation rooms; examination rooms; nursing assessment; preventive healthcare; maternal and child health; immunisation; counselling; telemedicine; treatment and minor procedures.
- **C. Urgent care:** triage; emergency assessment; resuscitation capability only if justified; treatment bays; observation spaces if justified; ambulance drop-off; emergency support areas.
- **D. Diagnostics:** sample collection; basic laboratory; imaging, if clinically and economically justified; diagnostic support and reporting.
- **E. Pharmacy:** dispensing; secure storage; temperature-controlled storage where required; staff work area.
- **F. Clinical support:** clean utility; dirty utility; clinical waste holding; medical supplies; equipment storage; housekeeping; linen storage; staff changing.
- **G. Building support:** electrical and backup power; water storage and treatment, if required; medical gases, if clinically required; IT and communications; mechanical plant; wastewater systems; general and biomedical waste management.
- **H. External areas:** ambulance bay; patient drop-off; accessible parking; staff parking, if appropriate; pedestrian access; service access; landscape; outdoor waiting or therapeutic spaces where suitable; future expansion zone.

For each department, explain why it is included, excluded or deferred. Do not include operating theatres, intensive care, major inpatient wards or specialised treatment facilities unless the service model, staffing, infrastructure, economics and regulatory requirements justify them.

10. Phase 6 — Detailed Architectural Area Programme

Create a comprehensive room-by-room schedule in square metres, distinguishing: net room area; departmental net area; internal circulation; walls and shafts; building support area; gross floor area; external site area.

For every space, include: (1) department; (2) room name; (3) number of rooms; (4) area per room; (5) total net area; (6) occupancy or users; (7) primary activities; (8) adjacency requirements; (9) privacy requirements; (10) accessibility requirements; (11) infection-control requirements; (12) water and drainage requirements; (13) electrical and IT

requirements; (14) HVAC requirements; (15) medical gas requirements, if applicable; (16) furniture and equipment; (17) regulatory or technical references; (18) design assumptions.

Calculate departmental subtotals and the total building area. Show the calculation of gross floor area from the net programme, explain any circulation, service or grossing allowances and avoid counting the same area twice.

Provide at least two scenarios: a compact, primary-care-led facility; and an expanded community facility, if the demand analysis supports it. Do not select an arbitrary total floor area before developing the programme.

Required deliverable. A detailed area schedule that can be transferred to a spreadsheet and used for subsequent architectural planning.

11. Phase 7 — Functional Relationships and Circulation

11.1 Adjacency analysis. Create a room and department adjacency matrix identifying: essential adjacency; preferred adjacency; separation requirements; controlled access; public and staff-only zones; clean and dirty workflows; emergency connections; service and delivery routes.

11.2 Bubble diagrams. Prepare at least three alternative functional concepts. Each must demonstrate: public zone; outpatient zone; urgent-care zone; diagnostic zone; pharmacy; staff and administration; clinical support; service access; future expansion. Explain the advantages and disadvantages of each.

11.3 Circulation planning. Develop separate movement studies for: outpatients; emergency patients; ambulances; staff; visitors; supplies; clean materials; soiled materials; biomedical waste; food and deliveries, if applicable. Provide route diagrams and explain where separation is essential and where shared circulation is acceptable. Do not claim complete circulation separation unless the actual plan demonstrates it.

Required deliverable. An adjacency matrix, three bubble diagrams, a preferred functional concept and circulation diagrams.

12. Phase 8 — Concept Development

Develop an architectural concept that responds to the verified site, clinical programme, local context and environmental conditions. Explore at least three design concepts, for example a courtyard-based facility, a compact pavilion or clustered clinical blocks, or a linear healthcare spine with controlled expansion. Treat these as options, not predetermined solutions.

For each concept, explain: design idea; site response; massing strategy; functional organisation; daylight and ventilation; patient experience; staff efficiency; landscape integration; structural implications; construction complexity; future expansion; cost implications. Select the preferred concept using an explicit evaluation matrix.

Design narrative. Write: (1) a one-sentence concept statement; (2) a short architectural narrative; (3) three to five design principles; (4) a diagrammatic explanation of how the concept translates into form; (5) a description of the patient arrival experience; (6) a description of the clinical workflow; (7) a description of the relationship between building and landscape. Avoid superficial metaphors that do not inform the actual design.

13. Phase 9 — Site Planning and Massing

Develop the proposed site plan using verified or clearly labelled assumed dimensions. Show: plot boundary; building footprint; entrances; ambulance access; patient drop-off; accessible pedestrian routes; parking, if justified; service

entrance; loading and waste handling; fire and emergency access; landscape; surface drainage strategy; retaining structures, if necessary; utility zones; outdoor patient spaces; future expansion area; north arrow; scale bar; key dimensions; site coverage; floor space calculations where applicable.

If no surveyed parcel is available, develop a hypothetical test-fit plan and state the assumed dimensions, slope and orientation prominently.

Massing studies. Develop at least three massing alternatives and compare: building height; footprint; solar exposure; views; circulation; privacy; construction feasibility; earthworks; environmental effects; future extension; site coverage. Explain the selected option.

14. Phase 10 — Floor Plan Development

Produce coordinated preliminary floor plans for every required level. For each, include: drawing title; scale; north arrow; grid or structural logic; room names and numbers; dimensions; room areas; door positions; windows; walls and partitions; corridors; accessible circulation; stairs and lifts, if required; sanitary facilities; furniture; clinical equipment; main entrances; emergency exits; departmental boundaries; relevant service spaces.

Provide: (1) ground-floor plan; (2) upper-floor plan, only if justified; (3) roof plan; (4) furniture and equipment layout; (5) functional zoning plan; (6) patient circulation plan; (7) staff circulation plan; (8) emergency circulation plan; (9) clean and dirty workflow diagrams; (10) expansion plan.

Make the floor plans consistent with the approved area schedule and room relationships. Do not generate arbitrary room arrangements without explaining how patients, clinicians, supplies and waste move through the building. For a small facility, evaluate a single-storey arrangement before proposing additional floors, considering terrain, accessibility, land availability, operational efficiency and expansion requirements.

If you cannot produce precise CAD drawings directly, provide dimensioned schematic plans, coordinates or a clearly specified drafting method suitable for implementation in CAD or BIM software. Do not misrepresent a conceptual diagram as a construction drawing.

15. Phase 11 — Elevations, Sections and Architectural Character

Elevations. Prepare descriptions or drawings for the main entrance, rear, side and service elevations, including: openings; shading; entrance canopy; material transitions; building proportions; accessibility; weather protection; signage; landscape relationship.

Sections. Prepare at least two important building sections demonstrating: ground relationship; slope response; floor-to-floor heights; ceiling and service zones; daylight; ventilation; roof drainage; structural logic; accessibility; relationships between clinical departments.

Architectural language. Develop a coherent material and façade strategy appropriate to Bogotá’s climate and the project’s maintenance budget. Evaluate: durable wall systems; weather-resistant external finishes; insulated roof and envelope options; high-performance windows; solar control; glazing privacy; acoustic control; easy-to-clean clinical finishes; locally available materials; maintenance and replacement cycles. Avoid specifying products or construction systems without confirming local availability, technical performance and compliance.

16. Phase 12 — Colombian Regulatory Compliance

Research the current applicable legal and technical framework. Verify, as relevant: seismic-resistant construction regulations, including the applicable NSR framework and amendments; planning and building licensing requirements; land-use and development restrictions; healthcare service provider registration and service-enablement requirements; healthcare infrastructure and equipment standards; accessibility; fire and life safety; occupational health and safety; environmental approvals; water supply and sanitation; biomedical waste management; medical gas systems; electrical installations; emergency evacuation; structural safety; local development and environmental constraints.

Investigate the current relevance and applicability of the applicable versions of: NSR-10 and subsequent amendments; Resolution 3100 of 2019 and subsequent amendments or replacement provisions; Resolution 4445 of 1996 and subsequent provisions, where relevant; RETIE and other applicable electrical regulations; relevant Colombian technical standards and municipal requirements. These are research starting points, not an assertion that every provision remains current or applies to the proposed facility.

For each relevant requirement, provide: regulation or standard; current version or status; issuing authority; relevant provision; design implication; verification source; professional responsible for confirmation.

Create a compliance matrix with the status: **Verified**, **Preliminary**, **Not assessed**, **Requires specialist confirmation**. Never declare the design legally compliant without the necessary technical reviews and approvals from qualified Colombian professionals and competent authorities.

17. Phase 13 — Structural and Geotechnical Strategy

Develop a preliminary structural concept informed by the site and building geometry. Investigate: local seismic design requirements; site-specific geotechnical conditions; soil bearing capacity; slope stability; groundwater; differential settlement; retaining structures; foundation alternatives; structural grid; column placement; long-span clinical spaces; service penetrations; roof structure; future expansion joints or connection strategy.

Compare feasible structural systems without inventing soil properties or final member sizes. Provide: preliminary structural concept; proposed structural grid; foundation options and selection criteria; structural coordination risks; required geotechnical investigations; required engineering calculations. State clearly that foundation sizing and structural design require qualified engineering analysis and site investigation.

18. Phase 14 — MEP and Clinical Building Systems

Develop a coordinated preliminary strategy for the building services.

- **Mechanical:** heating and thermal comfort; ventilation; outdoor air; filtration where required; pressure relationships in applicable clinical rooms; exhaust systems; humidity and condensation control; equipment maintenance access.
- **Electrical:** normal power; essential and emergency power; critical clinical circuits; lighting; emergency lighting; earthing and protection; backup generator or other justified backup systems; solar photovoltaic feasibility, if appropriate; electrical distribution.
- **Plumbing and sanitation:** potable water; hot water; water storage; water conservation; wastewater; stormwater; clinical handwashing; accessible toilets; backflow prevention; water quality monitoring.
- **Medical gases:** include only the systems justified by the selected clinical services. Determine oxygen, vacuum or other medical gas requirements from the clinical brief and applicable standards.

- **IT and communications:** network infrastructure; electronic health records; cybersecurity; telemedicine; nurse call systems where required; access control; CCTV subject to privacy requirements; communications resilience.

Equipment schedule. Show: equipment name; quantity; room; space requirement; utility requirements; maintenance access; indicative procurement considerations. Identify specialist engineering coordination requirements and avoid presenting conceptual service diagrams as installation-ready documents.

19. Phase 15 — Infection Control and Patient Safety

Develop a clinical planning strategy addressing: hand hygiene; clean and dirty workflows; isolation needs where justified; clinical waste; sterile supplies, if relevant; surface cleanability; ventilation requirements; patient privacy; medication security; safe staff circulation; slip and fall prevention; emergency access; patient dignity; accessibility; safeguarding vulnerable patients; emergency evacuation.

Differentiate infection-control recommendations from mandatory requirements. Coordinate the strategy with qualified healthcare professionals and infection-prevention specialists.

20. Phase 16 — Environmental Sustainability and Landscape

Develop a context-specific sustainability strategy. Evaluate: passive thermal design; envelope insulation; daylight without excessive heat loss or glare; solar shading; natural ventilation where clinically appropriate; energy-efficient mechanical systems; water conservation; rainwater harvesting, subject to local regulations and intended use; efficient lighting; renewable energy feasibility; construction waste reduction; durable materials; low-maintenance landscaping; biodiversity protection; stormwater management; erosion control; landscape-based patient wellbeing.

Do not use natural ventilation in spaces where clinical requirements demand controlled ventilation or pressure relationships.

Develop a landscape plan showing: existing vegetation; proposed planting; accessible outdoor spaces; shaded waiting; therapeutic gardens where suitable; pedestrian routes; stormwater treatment; ecological protection; maintenance requirements. Quantify projected energy or water savings only where a transparent calculation or credible model supports the estimate.

21. Phase 17 — Universal Accessibility and Wayfinding

Develop an inclusive design strategy for: wheelchair users; older adults; children; pregnant patients; people with sensory disabilities; people with temporary mobility restrictions; patients experiencing distress. Investigate applicable Colombian accessibility requirements.

Coordinate: step-free routes; ramps and gradients; door clearances; corridor widths; accessible sanitary facilities; reception counters; waiting areas; lifts where necessary; handrails; tactile and visual information; signage; emergency evacuation; drop-off points; accessible parking. Use actual regulatory dimensions only after verifying the applicable standard. Develop a wayfinding hierarchy that minimises confusion and makes the main services easy to locate.

22. Phase 18 — Preliminary Cost Estimation

Develop a transparent, preliminary project cost plan in Colombian pesos (COP). Research current local cost evidence where accessible and identify the price date and source.

Separate: (1) land acquisition; (2) surveys and investigations; (3) site preparation and earthworks; (4) foundations and structure; (5) building envelope; (6) interior construction and finishes; (7) mechanical and electrical systems; (8) clinical equipment; (9) furniture; (10) IT and communications; (11) landscape and external works; (12) utility connections; (13) professional fees; (14) permits and approvals; (15) contingency; (16) escalation and taxes where applicable; (17) commissioning and operational readiness.

State whether each figure is a sourced benchmark, calculated estimate or placeholder. Explain the cost basis, floor area basis, exclusions and uncertainty range. Do not invent precise local construction rates; if current reliable data are unavailable, show the cost model with clearly labelled input fields and request a local quantity surveyor's validation. Provide low, base and high scenarios only when their assumptions are explicit.

23. Phase 19 — Construction Strategy and Project Phasing

Develop a preliminary delivery strategy covering: feasibility and site verification; land acquisition and approvals; surveys and geotechnical investigations; clinical brief confirmation; concept design; schematic design; design development; technical coordination; tender documentation; procurement; construction; equipment installation; testing and commissioning; staff training; healthcare service registration and operational readiness; opening and post-occupancy evaluation.

Identify dependencies, critical approvals and major risks. Consider phased construction or future expansion only where clinically and economically appropriate. Do not provide an unrealistically precise completion date before the project scope, approvals and procurement strategy are established.

24. Phase 20 — Risk Register

For each risk, record: description; probability; impact; risk rating; evidence or reason; mitigation; responsible party; required investigation; decision deadline or project stage.

Include: unconfirmed healthcare demand; unverified site availability; land-use incompatibility; environmental restrictions; geotechnical instability; landslide and flood risks; road and ambulance access; water and power reliability; workforce availability; construction cost escalation; regulatory changes; clinical workflow failures; infection-control shortcomings; insufficient expansion capacity; operational funding; community acceptance; delays in approvals.

Clearly separate fatal constraints, major risks and manageable design issues.

25. Phase 21 — Architectural Drawing and Presentation Package

- **A. Research and feasibility:** regional location map; healthcare accessibility map; existing healthcare inventory; population and catchment analysis; site comparison matrix; environmental constraints map; feasibility conclusion.
- **B. Concept design:** client objectives; design brief; concept alternatives; concept selection matrix; bubble diagrams; adjacency matrix; functional zoning; circulation strategy; massing alternatives.

- **C. Architectural drawings:** site plan; ground-floor plan; upper-floor plans, if applicable; roof plan; elevations; building sections; landscape plan; furniture layouts; clinical equipment layouts; accessibility diagrams; emergency access diagrams; expansion strategy.
- **D. Technical strategy:** structural concept; MEP strategy; medical gas strategy where applicable; fire and life safety; infection prevention; sustainability; material palette; regulatory compliance matrix.
- **E. Visualisation:** exterior entrance perspective; main building exterior; aerial massing view; reception and waiting area; typical consultation room; courtyard or outdoor patient space, if included; representative building section perspective.
- **F. Project summary:** recommended facility type; proposed clinical services; total net area; gross floor area; plot area and status; number of floors; preliminary construction cost range, if supportable; key sustainability strategies; major risks; outstanding approvals; next steps.

Visualisations must be consistent with the actual floor plans, elevations, massing and site assumptions. Do not depict unverified surrounding buildings, terrain or infrastructure as factual site conditions. For each drawing, specify the title, purpose, scale or intended presentation scale, essential information and required inputs. If image-generation tools are available, generate conceptual visuals only after the architectural brief and selected concept have been established, and treat them as design illustrations, not construction documentation.

26. Phase 22 — Final Report Structure

Organise the final proposal as follows:

1. Cover page
2. Executive summary
3. Project background
4. Client objectives
5. Project scope
6. Healthcare demand and service gap
7. Existing healthcare network
8. Site-selection methodology
9. Candidate site comparison
10. Site analysis
11. Climate and environmental response
12. Healthcare service model
13. Architectural brief
14. Detailed area programme
15. Functional adjacency
16. Concept alternatives
17. Selected design concept
18. Site planning and massing
19. Floor plans
20. Elevations and sections
21. Material and façade strategy
22. Structural concept
23. MEP and clinical systems
24. Infection prevention and patient safety
25. Accessibility
26. Sustainability and landscape

27. Regulatory compliance
28. Preliminary cost plan
29. Construction phasing
30. Risk register
31. Limitations and outstanding investigations
32. Recommendations
33. References and appendices

Use professional architectural language, numbered headings, concise technical tables, explanatory diagrams and traceable references. Do not inflate the report with repetitive generic descriptions.

27. Quality Control and Consistency Audit

Before delivering the final proposal, verify that:

1. The selected facility type responds to documented demand.
2. The site-selection recommendation is evidence-based.
3. No unverified parcel is presented as a confirmed site.
4. The room schedule matches the floor plans.
5. Departmental areas add up correctly.
6. Net and gross floor areas are clearly distinguished.
7. Patient and staff workflows are coherent.
8. Emergency access is feasible.
9. Accessibility is addressed.
10. Clinical support spaces are included where necessary.
11. Structural and MEP strategies match the architectural design.
12. Infection-control needs are reflected in planning.
13. Climate and terrain inform the building design.
14. Expansion provisions are realistic.
15. Cost estimates have a traceable basis.
16. Regulatory statements are current or explicitly marked for verification.
17. References support the claims made.
18. Drawings do not contradict one another.
19. Assumptions and uncertainties are visible.
20. The proposal does not claim construction readiness without the necessary detailed engineering, surveys and approvals.

Provide a final design-coordination checklist identifying unresolved inconsistencies.

28. Required Working Method

Work sequentially rather than generating an entire unverified hospital proposal in one step.

- **Stage 1 — Evidence and feasibility:** project definition, source research, healthcare accessibility assessment, site-selection criteria and preliminary service model.
- **Stage 2 — Design brief:** clinical capacity, departmental requirements, room schedule, area calculations and adjacency requirements.
- **Stage 3 — Concept design:** three concepts, compared, with one recommended.
- **Stage 4 — Schematic design:** site planning, massing, floor plans, circulation, sections and elevations.

- **Stage 5 — Technical coordination:** structural, MEP, accessibility, infection-control, sustainability and regulatory strategies.
- **Stage 6 — Cost and delivery:** cost model, project phasing, risk register and implementation recommendations.
- **Stage 7 — Final presentation:** report, drawing list, visualisation brief, compliance matrix and design audit.

At the end of each stage: summarise the decisions made; list evidence used; list assumptions; identify missing inputs; identify decisions requiring client approval; identify specialist investigations; state whether the next stage can proceed responsibly.

Do not silently carry an unverified assumption into later stages as if it were established fact. If critical information is missing, continue with a clearly labelled preliminary scenario where possible, but do not fabricate evidence.

29. Your First Response

Begin by producing Stage 1 only. Your first response must contain:

1. A concise project understanding.
2. A provisional client objective.
3. A research and evidence plan.
4. A preliminary healthcare service-gap assessment framework.
5. A list of existing healthcare facilities and relevant datasets to investigate, with sources where available.
6. A site-selection methodology for Pasquilla–Mochuelo.
7. A preliminary regulatory research checklist.
8. A table of known information, assumptions and missing data.
9. The critical questions that must be answered before selecting a parcel.
10. A clear recommendation on the next action.

Do not invent a site, population, hospital capacity, cost or final floor area. After Stage 1, wait for approval or new information before proceeding to Stage 2.

The ultimate objective is to create a coherent, evidence-based, locally appropriate and technically coordinated architectural proposal that can be developed further by qualified Colombian architects, healthcare planners and engineers.